

NNP Pocket Guide 2

FEN, CV, and Resp
Version 1, October 2026
Companion to the procedures booklet; same ring.

Tags

- Course
- N5537 lectures and handouts (Dr. Short), and your study guides built from them
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- Card
- Your instructor's badge card
-
- Dir.
- Program director's NNP Brain
-
- AAP
- AAP 2022 hyperbilirubinemia guideline
-
- Sources disagree, or an error was found

Sections

- FEN: GIR, TPN, calories, bilirubin
- CV: pressors, profiles, BP norms
- Resp: blood gas, vent
- Ref: sources

Starting points, not orders. Follow unit protocol and your attending.

TPN: start and advance

When: <32 wk, VLBW/ELBW, GI anomaly or surgery, NPO >3–5 days.

Course

Day 1: D10 at 60–80 mL/kg/d, AA 2–3 g/kg/d, lipid 1 g/kg/d.

Course

Amino acids	Within 24 h. AA slide: start 3, +0.5/d to 3–4, max 4. Course Card: start 2–3, goal 3–4. Card
Lipid	1–2 → 3 g/kg/d. 0.5–1 prevents EFAD. TG >200: stop or cut 0.5–1. Course
Sodium	None first 24 h, then 2–4 Course Card: 1–2 from day 1, goal 3–4 Card
Potassium	None first 24–48 h, then 1–3 once urine output is established Course Card: 1–2 from day 1, goal 2–4 Card
Ca gluconate	200–400 mg/kg/d Course Card

The lecture writes Na and K as mEq/d; they're usually written per kg per day.

Peripheral TPN under 900 mOsm/L.

Course

Bilirubin

35 weeks and up

AAP

Thresholds are hour-specific curves by GA and risk: plot them in BiliTool or PediTools.

Neurotoxicity risk factors: GA <38 wk, albumin <3.0, hemolysis (isoimmune, G6PD), sepsis, instability in the past 24 h.

Escalation: TSB within 2 of the exchange line = emergency. NICU, intensive phototherapy, PO + IV fluids, STAT labs, TSB every 2 h.

Stop phototherapy at ≥2 below the start threshold; rebound in ~24 h (6–12 h if started before 48 h).

Under 35 weeks (Maisels 2012)

PMA	Photo	Exchange
<28 wk	5–6	11–14
28–29	6–8	12–14
30–31	8–10	13–16
32–33	10–12	15–18
34–35	12–14	17–19

TSB mg/dL. Born <35 wk: stay on the top row after 35 wk PMA.

Pressor doses

Drug	Dose	Use
Dopamine	≤2 / ≥5–10 / ≥10 mcg/kg/min	DA → β1 → α1 with dose. ≥10: R→L ductal shunt risk. Not with PPHN.
Dobutamine	≥3–15 mcg/kg/min	β1 inotrope, mild β2 dilation. Avoid in outflow obstruction (IDM).
Epinephrine	0.02–0.1, then ≥0.1 (caution >0.2)	Low dose β, high dose α1. Lactate, glucose, tachycardia.
Norepinephrine	0.02–0.4 mcg/kg/min	α1 > β1. Severe vasoplegia.
Vasopressin	0.0002–0.005 units/kg/min	V1a. Catecholamine-resistant; spares PVR. Low Na, low platelets.
Milrinone	0.25–0.75 mcg/kg/min, no load	PDE3 inodilator. Low output + PH, post-ligation. Onset 1–2 h; drops BP.
Hydrocortisone	1 mg/kg IV q8h	Adrenal insufficiency, or refractory on ≥2 agents.

Practice-question guide uses dopamine tiers 1–5 / 5–10 / >10.

Course

TPN calories

Glucose	3.4 kcal/g Course Dir. (card: carbohydrate 4)
Protein	4 kcal/g
Fat	9 kcal/g
20% lipid	1 g = 5 mL = 10 kcal (2 kcal/mL) Course

Total = GIR × 5 + AA g/kg × 4 + lipid g/kg × 10 [Course](#)

Exact dextrose kcal/kg/day = %D × mL/kg/day × 0.034 (GIR × 4.9).

Lipid g/kg/day = mL/hr × 24 × 0.2 ÷ kg [Course](#)

Example: 1.8 kg, D12 at 10.2 mL/hr, AA 3.5, lipid 1.1 mL/hr ≈ 99 kcal/kg/day.

Goals [Course](#)

Preterm 110–130, term 100–120 kcal/kg/day. Gain 15–20 g/kg/day.

Feeds: kcal/mL = kcal/oz ÷ 30

20 → 0.67	22 → 0.73	24 → 0.8
26 → 0.87	28 → 0.93	30 → 1.0

FEN

G2-4

Decision tree and algorithm

- Bed loose** (warm, low diastolic) → norepinephrine (or dopamine)
- Pump weak** (cool, poor contractility) → dobutamine
- Pump weak + high afterload** (PPHN, post-op) → milrinone, add a pressor if BP sags
- Both failing** → epinephrine
- Still low on catecholamines** → vasopressin; think hydrocortisone

Around it [Course](#)

Perfusion adequate (NIRS, urine output, lactate, echo)? Then permissive hypotension. Reassess after every change; BP up with worse perfusion means over-constriction. Wean catecholamines first, keep milrinone while PH persists, wean vasopressin last.

▶ The lecture defaults to dopamine first for vasoplegia; much current practice starts phenotype-specific (e.g., norepinephrine or epinephrine in septic shock). Practice question 5 keys dopamine for high-SVR cold shock against the lecture's own afterload logic. Know the keyed answers; follow your unit.

CV

G2-8

GIR (mg/kg/min)

Shortest	%D × mL/kg/day ÷ 144 Course
Hourly	%D × mL/hr ÷ (6 × kg) Course Dir. = %D × mL/hr × 0.167 ÷ kg Card
Long way	mL/day × %D (decimal) = g; × 1000 = mg; ÷ kg ÷ 1440 Course

%D for a target GIR = GIR × 144 ÷ mL/kg/day

Example: D10 at 80 mL/kg/day = GIR 5.6.

Targets

Start 4–6, then titrate to glucose; preterm need at least 4.

[Course](#)

Target 4–8. [Card](#)

Intro lecture: 2–4 most newborns, 8–10 SGA/LGA, >15 IDM.

[Course](#)

Max dextrose [Course](#)

12.5% peripheral. 25% central with the tip in the SVC.

FEN

G2-2

Pick the pressor

BP = CO × SVR: pipes, pump, or both?

Low diastolic	Pipes (low SVR) → vasopressor (α1 or V1)	
Low systolic	Pump (low contractility) → inotrope (β1 or PDE3)	
Both	Cold, mottled, acidotic → both actions	
	Warm (low SVR)	Cold (high SVR)
Skin	Warm, pink	Cool, mottled
Refill	Flash <2 s	>3 s
Pulses	Bounding	Thready
Pulse pressure	Wide	Narrow
Causes	Sepsis, NEC, big PDA	Cold sepsis, transitional, post-ligation

Hypotensive, warm, and bounding is **not** reassuring: it's vasodilated and needs tone. [Course](#)

CV

G2-6

Hemodynamic profiles

Read diastolic BP, pulse pressure, perfusion index, SVR, cardiac output, and StO₂ together. [Course](#)

Profile	Pattern
Vasodilatory	↓ DBP, ↓ SVR; ↑ PP, PI, CO (warm shock)
Vasoconstrictor	↑ DBP, ↑ SVR; ↓ PP, PI, CO (cold shock)
Cardiogenic	↓ CO, low PP, PI, DBP
L-R shunt	↑ CO, ↑ PP; ↓ DBP, ↓ SVR (PDA run-off)
Volume depletion	↓ CO, ↓ PP; ↑ SVR

Same low diastolic, opposite fixes: vasodilatory needs a vasoconstrictor; volume-depleted needs volume.

Epinephrine 0.2 mcg/kg/min for an hour ≈ one resuscitation bolus (10 mcg/kg).

CV

CV G2-9

Read a blood gas

1. **pH**: <7.35 acidemic, >7.45 alkalemic
2. **PaCO₂**: respiratory?
3. **HCO₃⁻ / BE**: metabolic?
4. **Compensation, then the bedside picture**
CO₂ opposite pH → respiratory. HCO₃⁻ with pH → metabolic.

pH	7.35–7.45 (>7.30 may be OK in context)
PaCO₂	35–45
HCO₃⁻	22–26
BE	–2 to +2; worse than –4 to –6, watch

Uncompensated	pH off, other value normal
Partial	pH off, both abnormal
Compensated	pH normal, both abnormal

Lungs fix CO₂ quickly; kidneys fix HCO₃⁻ over days. High CO₂ and low HCO₃⁻ = mixed acidosis, not compensation. [Course](#)

Resp

Resp G2-11

Vent modes

AC	Every breath supported. Poor effort, apnea, extreme prematurity.
SIMV	Synced breaths plus spontaneous. Weaning, low support, sedation backup.
Volume guarantee	Hits a set volume; unstable compliance; guards against volutrauma.
Pressure control	Caps pressure; volume varies. RDS, severe BPD.
HFOV	Open lung, tiny volumes. PIE, air leak, PPHN.
HFJV	Fast small bursts; conventional vent gives PEEP. PIE, refractory CO ₂ .

By disease [Course](#)

RDS: CPAP → PC or volume-targeted; HFOV if severe. **TTN**: HFNC or CPAP. **BPD**: long expiratory time, permissive CO₂, often VG. **PIE/air leak**: HFJV or HFOV. **Pulmonary hemorrhage**: higher MAP. **PPHN**: conventional or HFOV.

Resp

Resp G2-13

Jet, weaning, and OI

HFJV

Ventilation: PIP above PEEP (ΔP). Oxygenation: PEEP (+1–2 when FiO₂ ≥ 0.6). Don't wean PEEP until FiO₂ < 0.35–0.40.

HFOV weaning [Dir.](#)

- PaCO₂ < 50 and pH > 7.25: wean amplitude
- FiO₂ < 70%, PaO₂ > 60, good inflation: wean MAP
- Extubation: term MAP 11–14, amp ≤ 20; LBW MAP 10–12, ≤ 15; VLBW MAP < 10, < 12
- Air leak: wean MAP hard, smallest amplitude, accept CO₂ high 50s–low 60s, PIE side down

OI = MAP × FiO₂ × 100 ÷ PaO₂ [Dir.](#)

OI > 40 for > 2 h, or in 3 of 5 gases: 80% predicted mortality (director's sheet).

Resp

Resp G2-15

What moves what

CO ₂ ↑	pH ↓ (respiratory acidosis)
CO ₂ ↓	pH ↑ (respiratory alkalosis)
HCO ₃ ⁻ ↓ (lactate, TPN acid, RTA, diarrhea)	pH ↓, BE more negative
HCO ₃ ⁻ ↑ (vomiting, diuretics, bicarb, acetate)	pH ↑
Bicarb given	HCO ₃ ⁻ + H ⁺ → carbonic acid → CO ₂ + water: CO ₂ rises; pH can fall if not ventilated
CO ₂ high for days	Kidneys keep HCO ₃ ⁻ → pH toward normal
Metabolic acidosis	Lungs blow off CO ₂ quickly
Rate or tidal volume ↑	CO ₂ ↓, pH ↑
MAP or FiO ₂ ↑	Oxygenation ↑

Bicarb: only with adequate ventilation; not in the same line as catecholamines; can lower K.

▶ The Pat Thomas guide says kidneys "retain H⁺" in respiratory acidosis; they excrete H⁺ and keep HCO₃⁻.

Sources

Course: Dr. Kira Short's N5537 Intro to Neonatal FEN and Neonatal TPN lectures; FEN formulas handout; chorio and EOS handout. Your study guides: Cardiovascular Function & Treatments; Choosing a Pressor; Neonatal Hemodynamics; Module 2 respiratory; Blood Gas Interpretation (Pat Thomas).

Card: your instructor's badge card. **Dir.:** program director's NNP Brain.

Bilirubin: AAP 2022 Clinical Practice Guideline (Kemper et al.); Maisels et al., J Perinatol 2012; UNC NCCC hyperbilirubinemia guideline (2025).

BP: Dionne, Abitbol, Flynn 2012 (erratum) via Batton, Clin Perinatol 2020; Starship and Medscape neonatal hypertension.

Vent: Loyola neonatal ventilation supplement; SPA oscillator review; UNC NCCC high-frequency ventilation; UW Madison NICU jet guideline; NHS GGC HFOV.

Bicarb: Canterbury DHB and NSW ANMF neonatal guidelines.

The full versions, calculators, and links are in the NNP app.

BP norms (after 2 weeks)

By PMA; 50th / 95th / 99th percentile, mmHg (Dionne 2012, corrected table).

PMA	MAP	Systolic	Diastolic
44	63/80/85	88/105/110	50/68/73
42	62/76/81	85/98/102	50/65/70
40	60/75/80	80/95/100	50/65/70
38	59/74/79	77/92/97	50/65/70
36	57/72/77	72/87/92	50/65/70
34	50/65/70	70/85/90	40/55/60
32	49/64/69	68/83/88	40/55/60
30	48/63/68	65/80/85	40/55/60
28	45/58/63	60/75/80	38/50/54
26	38/57/63	55/72/77	30/50/56

High BP: ≥95th on 3 occasions; >99th, investigate. **"MAP ≥ GA":** weak evidence; judge with perfusion. **Cuff:** compare MAP (the measured value), right arm.

What to change: conventional and HFOV

Conventional

CO ₂ high	Rate +5–10; PIP +2 for chest rise; lower PEEP if over-expanded
O ₂ low	FiO ₂ ↑; PEEP ↑; PIP +1–2; Ti 0.4–0.6 s
Weaning	FiO ₂ to 80% before changing PIP, I:E, PEEP; steps PIP 2, FiO ₂ 5%, rate 5

$$\text{MAP} = (\text{PIP} - \text{PEEP}) \times \text{Ti} \div (\text{Ti} + \text{Te}) + \text{PEEP}$$

HFOV

CO ₂ high	Amplitude ↑; lower Hz if needed
CO ₂ low	Amplitude ↓; raise Hz
O ₂ low	MAP ↑; FiO ₂ ↑
O ₂ high	FiO ₂ ↓, then MAP ↓

Lower Hz = bigger volumes, more CO₂ removal; Hz changes are usually a senior call.